

EMS Scenarios - Data Collection - Spring 2025

Scenario 1 - - Protocol Chest Pain - Cardiac Suspected

En Route

Dispatch: Medic 425 is responding to a **58-year-old male** experiencing **chest pressure and shortness of breath** at his office. Caller reports **he has a history of hypertension and high cholesterol**.

On Scene

Coworker: *He said his chest felt heavy, like someone was sitting on it. He looked pale and started sweating, so we called 911 right away.*

[EMT and Paramedic approach the patient, who is seated, holding his chest, appearing pale and diaphoretic.]

EMT: *"Sir, my name is [Name]. We're here to help. Can you tell me what you're feeling?"*

Patient: *"Tight... pressure in my chest... started about 15 minutes ago."*

[EMT assesses ABCs. Airway and breathing intact, but patient appears anxious and SOB.]

[Pulse oximeter shows SpO₂ = 93%. Oxygen is applied via nasal cannula at 4 LPM.]

EMT: *"We're going to give you some oxygen to help with your breathing."*

EMT:

- *"Do you have a history of heart problems?" → "Yes, high blood pressure and borderline cholesterol."*
- *"Are you on any medications?" → "Yes, lisinopril and aspirin."*
- *"Have you taken anything like Viagra or Cialis in the last 48 hours?" → "No."*

[Paramedic places patient on cardiac monitor. 12-lead ECG shows ST elevations in II, III, and aVF (Inferior STEMI).]

EMT: *"This ECG shows signs of a possible heart attack. We need to get you to the hospital quickly."*

EMT (Radioing Hospital):

"Medic 425 en route with a 58-year-old male, chest pressure, 12-lead indicates ACUTE MI (Inferior STEMI). Initiating STEMI alert to [Nearest PCI Center]. ETA 12 minutes."

EMT: *"We're going to give you some aspirin to chew. It helps reduce damage to your heart."*

[Administers 324 mg chewable aspirin.]

Patient: *"I don't carry nitro, but I've had it in the hospital before."*

[Since BP is 138/84 mmHg, EMT administers 0.4 mg nitroglycerin sublingually.]

EMT: *"This medication should help relieve the pressure. Let us know if anything changes."*

[After 5 minutes, pain persists. Second dose of nitro is administered.]

[Patient is secured and transported in position of comfort. EMTs continue monitoring ECG and vitals.]

Vitals:

- **BP:** 138/84 mmHg
- **Pulse:** 96 bpm
- **Respirations:** 20/min
- **SpO₂:** 95% on oxygen

Scenario 2 - - Protocol Chest Pain - Cardiac Suspected

En Route

Dispatch: Medic 425 is responding to a **62-year-old female** experiencing **nausea, shortness of breath, and upper back discomfort** while gardening. Caller reports the patient has a **history of type 2 diabetes and hypertension**.

On Scene

Neighbor: *She suddenly stopped what she was doing, grabbed her back, and said she felt nauseous and dizzy. She has heart problems, I think.*

EMT: *"Ma'am, what are you feeling?"*

Patient: *"I don't know... I feel sick, like I'm going to throw up... and my back really hurts."*

[EMT evaluates ABCs – Airway clear, breathing fast but adequate, circulation intact.]

[SpO₂ reads 91%. Oxygen administered via nasal cannula at 4 LPM to raise saturation above 94%.]

EMT: *"We're going to give you some oxygen and check your heart."*

[EMT applies monitor leads and performs a 12-lead ECG within 10 minutes.]

[ECG does not show classic STEMI but reveals nonspecific T-wave changes.]

EMT: *"Your heart rhythm isn't showing a full heart attack, but your symptoms are still very concerning. Let's get you to the hospital quickly."*

EMT (Radioing hospital):

"Medic 425 en route with a 62-year-old female. Diabetic, hypertensive, with atypical chest discomfort and nausea. ECG shows T-wave abnormalities but no STEMI. ETA 10 minutes."

EMT: *"Please chew this aspirin—it helps prevent worsening heart damage."*

[Administers 324 mg aspirin to chew.]

Patient: *"I don't take nitro."*

[BP remains > 100 systolic. EMT administers 0.4 mg SL nitroglycerin under medical direction.]

EMT: *"This may help with the discomfort in your back."*

[Patient is placed in a semi-Fowler's position and secured in the ambulance. Continuous cardiac and vital sign monitoring is maintained.]

[Pain persists. Second dose of nitroglycerin is administered after 5 minutes, as BP remains stable.]

EMT: *"We're almost there. Let us know if anything changes."*

Scenario 3 - - Protocol Stroke

En Route

Dispatch: Medic 425 is responding to a 65-year-old female at a local grocery store, experiencing sudden confusion and weakness on one side of her body. Bystanders report slurred speech and difficulty walking.

On Scene

Store Employee: Please, over here! She was shopping when she suddenly dropped her bag and started leaning to one side. She's trying to talk, but her words don't make sense.

[Store employee leads the Paramedic and EMT to find the woman sitting on the floor, appearing confused and struggling to move her right arm.]

EMT: Ma'am, can you hear me? What's your name?

Patient: [Slurred speech] I... I don't know...

EMT: Let's assess her for a stroke.

[EMTs perform a rapid stroke assessment using the BEFAST criteria.]

- Balance: Ma'am, can you try to stand up?
 - *Patient attempts but is unstable and leans to the right.*
- Eyes: Any sudden vision problems?
 - *Patient blinks and shakes head slightly.*
- Face: Ma'am, can you smile for me?
 - *Right side of her face droops.*
- Arms: Can you raise both arms for me?
 - *Patient's right arm drifts downward.*
- Speech: Can you repeat after me: "The sky is blue."
 - *Patient slurs words, unable to repeat phrase correctly.*
- Time: Symptoms started about 10 minutes ago, per bystander report.

EMT: She's showing clear signs of a stroke. Let's check her vitals and prep for transport.

[EMT takes vitals: BP 180/95 mmHg, pulse 88 bpm, respiration 16/min, SPO2 97%.]

EMT: We need to transport her immediately to a stroke center. Let's notify the hospital and activate the stroke protocol.

[EMT and Paramedic place the patient on a stretcher and load her into the ambulance.]

Scenario 4 - ODEMSA guidelines - Protocol Stroke

En Route

Dispatch: Medic 425 is responding to a 68-year-old female at a community center, experiencing sudden right-sided weakness and slurred speech. Symptoms began approximately 15 minutes ago.

On Scene

Bystander: *She was fine earlier, but suddenly she couldn't move her right arm and started speaking strangely.*

[Bystander leads the EMT and Paramedic to the patient sitting in a chair, appearing alert but distressed.]

Step 1: Perform General Patient Management

EMT: *Ma'am, my name is [Name]. We are here to help. Can you tell me your name?*

Patient: *[Slurred speech] Sarah...*

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT ensures the airway is open, checks for signs of aspiration, obstruction, or hypoventilation.]

EMT: *She has no airway obstruction. Breathing is present and adequate.*

Step 3: Administer Oxygen (if needed)

[EMT places a pulse oximeter on the patient's finger. SpO₂ reads 97%. No supplemental oxygen needed.]

EMT: *Her SpO₂ is within range. No oxygen required at this time.*

Step 4: Perform Cincinnati or FAST Stroke Scale & VAN Assessment

[EMT performs the **Cincinnati Prehospital Stroke Scale (CPSS)**]

- **Facial Droop:** *"Ma'am, can you smile for me?"*
 - *Right side of her face droops.*
- **Arm Drift:** *"Can you raise both arms?"*
 - *Right arm drifts downward.*
- **Speech:** *"Say, 'The sky is blue.'"*
 - *Speech is slurred and words are incorrect.*

[CPSS is positive, so EMT proceeds with **VAN (Vision, Aphasia, Neglect) Assessment**]

- **Vision Loss:** *"Do you see my fingers clearly?"*
 - *Patient struggles to see fingers on the right side.*
- **Aphasia (Speech Comprehension):** *"Can you name this object?" [Shows a pen]*
 - *Patient struggles to name it.*

- **Neglect:** *"Touch your left arm with your right hand."*
 - *Patient does not respond to command.*

[Positive VAN test confirms possible large vessel occlusion stroke.]

Step 5: Determine Last Known Well Time

EMT: *"When was the last time you saw her normal?"*

Bystander: *"About 15 minutes ago."*

Step 6: Notify the Hospital of Stroke Alert

EMT (Radioing hospital):

"Medic 425 en route with a 68-year-old female, positive for stroke (FAST & VAN positive). Symptoms onset ~15 minutes ago. BP 185/100. ETA 10 minutes. Stroke alert activated."

Step 7: Ensure a Witness Accompanies the Patient

EMT: *"Who was with her when symptoms started?"*

Bystander: *"I was."*

EMT: *"Please come with us to provide details to the doctors."*

In the Ambulance

Step 7: Perform Rapid Glucose Check

[EMT checks blood glucose: **Glucose = 95 mg/dL** (normal)]

EMT: *Blood sugar is normal. Stroke remains primary concern.*

Scenario 5 - ODEMSA guidelines - Protocol Stroke

En Route

Dispatch: Medic 425 is responding to a **72-year-old female at a library**, experiencing **sudden right-sided weakness and slurred speech**. Symptoms began approximately **20 minutes ago** per the caller.

On Scene

Bystander: *She was reading when she suddenly dropped her book and started mumbling. She's confused and can't move her right arm.*

[Bystander leads EMT and Paramedic to the patient, who is sitting in a chair, appearing confused.]

Step 1: Perform General Patient Management

EMT: *"Ma'am, my name is [Name]. We are here to help. Can you tell me your name?"*

Patient: *[Slurred speech] Uh...uh...*

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT checks airway, breathing, and circulation for any compromise.]

EMT: *"Airway is clear. Breathing is adequate. No signs of aspiration or obstruction."*

Step 3: Administer Oxygen (if needed)

[EMT places a pulse oximeter—SpO₂ 96% (no oxygen required).]

EMT: *"Oxygen levels are good. No supplemental O₂ needed."*

Step 4: Perform FAST Stroke Scale

[EMT performs FAST assessment for stroke evaluation.]

- **Face (F):** *"Ma'am, can you smile for me?"*
 - *Right side of face droops.*
- **Arms (A):** *"Can you raise both arms?"*
 - *Right arm drifts downward.*
- **Speech (S):** *"Repeat after me: 'The sun is shining today.'"*
 - *Speech is slurred and words are incorrect.*
- **Time (T):** *"When was the last time she was seen normal?"*
 - *Bystander: "About 20 minutes ago."*

[FAST is positive—proceeding with VAN assessment.]

Step 5: Perform VAN (Vision, Aphasia, Neglect) Test

- **Vision:** *"Can you see both sides of my fingers?"*
 - *Patient struggles to see on the right side.*
- **Aphasia:** *"What is this?" [Shows pen]*
 - *Patient struggles to name it.*
- **Neglect:** *"Touch your left hand with your right."*
 - *Patient does not acknowledge command.*

[Positive VAN test → Possible Large Vessel Occlusion Stroke]

Step 6: Notify the Hospital of Stroke Alert

EMT (Radioing hospital):

"Medic 425 en route with a 72-year-old female, positive for stroke (FAST & VAN positive). Symptoms started ~20 minutes ago. BP 190/102. Stroke alert activated. ETA 10 minutes."

Step 7: Ensure a Witness Accompanies the Patient

EMT: *"Who was with her when this happened?"*

Bystander: *"I was sitting next to her."*

EMT: *"Please come with us to provide details to the hospital."*

[Witness boards the ambulance or provides a contact number for hospital staff.]

In the Ambulance

Step 8: Perform Rapid Glucose Check

[EMT checks blood glucose: **Glucose = 82 mg/dL** (normal)]

EMT: *"Blood sugar is normal. Stroke remains the primary concern."*

Scenario 6 - ODEMSA guidelines - Protocol Stroke

En Route

Dispatch: Medic 425 is responding to a **74-year-old male at a coffee shop**, experiencing **sudden confusion and weakness in his left arm and leg**. Witnesses report **slurred speech and difficulty standing**. Symptoms started **approximately 10 minutes ago**.

On Scene

Bystander: *He was ordering coffee when he suddenly started slurring his words and nearly fell. We helped him to a chair, but he seems very confused and weak on one side.*

[EMT and Paramedic approach the patient, who appears alert but disoriented.]

Step 1: Perform General Patient Management

EMT: *"Sir, my name is [Name]. We're here to help. Can you tell me your name?"*

Patient: *[Slurred speech] Uh... mmm...*

[EMT evaluates responsiveness and mental status.]

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT checks for aspiration, upper airway obstruction, and hypoventilation.]

EMT: *"Airway is clear. Breathing is adequate, but let's keep monitoring."*

[Patient is showing no signs of respiratory distress.]

Step 3: Administer Oxygen (if needed)

[EMT places a pulse oximeter: SpO₂ 95%—no oxygen required.]

EMT: *"Oxygen saturation is within normal range. No oxygen needed at this time."*

Step 4: Perform FAST Stroke Scale

[EMT performs a FAST assessment.]

- **Face (F):** *"Sir, can you smile for me?"*
 - *Left side of his face droops.*
- **Arms (A):** *"Can you lift both arms?"*
 - *Left arm drifts downward.*
- **Speech (S):** *"Say, 'I love my family.'"*

- *Speech is slurred and incomprehensible.*
- **Time (T):** *"When was he last seen normal?"*
 - *Bystander: "About 10 minutes ago."*

[FAST is positive → Proceeding to VAN assessment.]

Step 5: Perform VAN (Vision, Aphasia, Neglect) Test

- **Vision:** *"Can you see my fingers clearly on both sides?"*
 - *Patient struggles to see on the left side.*
- **Aphasia:** *"Sir, what is this?" [Shows a key]*
 - *Patient is unable to name the object.*
- **Neglect:** *"Can you touch your right hand with your left?"*
 - *Patient does not acknowledge his left hand.*

[Positive VAN test → Possible Large Vessel Occlusion Stroke.]

Step 6: Notify the Hospital of Stroke Alert

EMT (Radioing hospital):

"Medic 425 en route with a 74-year-old male, positive FAST and VAN stroke assessment. Left-sided weakness, facial droop, slurred speech. Last known well time: ~10 minutes ago. BP 178/96. Stroke alert activated. ETA 8 minutes."

In the Ambulance

Step 7: Perform Rapid Glucose Check

[EMT checks blood glucose: Glucose = 88 mg/dL (normal)]

EMT: *"Blood sugar is normal. Stroke remains the priority."*

Step 8: Ensure a Witness Accompanies the Patient

EMT: *"Who was with him when this started?"*

Bystander: *"I was standing next to him."*

EMT: *"We need you to come with us or provide a contact number for the hospital."*

[Witness boards the ambulance or gives contact details.]

Scenario 7 - ODEMSA guidelines - Opioid Overdose

En Route

Dispatch: Medic 425 is responding to a **35-year-old male found unresponsive in a parking lot**. A bystander reported shallow breathing and pinpoint pupils.

On Scene

Bystander: *I saw him sitting in his car earlier, and when I walked by again, he was slumped over. I tried to wake him, but he wouldn't respond. I think he uses drugs.*

[EMT and Paramedic approach the patient, who is unresponsive and has shallow breathing.]

Step 1: Perform General Patient Assessment

EMT: *"Sir! Can you hear me?"* **[Shakes shoulders, no response]**

[Patient is checked for pulse and breathing—pulse present, but breathing is slow and shallow.]

EMT: *"We have an unresponsive patient with inadequate respirations. Possible opioid overdose."*

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT opens airway and performs head-tilt/chin-lift maneuver. Suctions oropharynx if needed.]

EMT: *"Patient has a weak gag reflex, airway is clear, but breathing is shallow. We need to assist ventilations."*

[Bag-Valve Mask (BVM) is applied at 10-12 breaths per minute with supplemental oxygen.]

EMT: *"SpO₂ is 88%. Let's administer oxygen to maintain 94-99% saturation."*

Step 3: Obtain Blood Glucose Sample

[EMT checks blood glucose: 98 mg/dL (normal). Hypoglycemia is ruled out.]

EMT: *"Blood sugar is normal. Overdose remains our primary concern."*

Step 4: Administer Naloxone (Narcan)

[EMT prepares Narcan for administration.]

EMT: *"Patient is still not breathing adequately. Suspected opioid overdose. Administering 2 mg Narcan intranasally."*

[Narcan is administered via nasal spray. EMT continues ventilating with BVM.]

[Patient remains unresponsive. After 2 minutes, another 2 mg dose of Narcan is given.]

[Patient begins to breathe more effectively and starts to regain consciousness.]

Step 5: Monitor and Prepare for Transport

EMT: *"Sir, can you hear me? You overdosed, but we gave you Narcan. We're taking you to the hospital."*

[Patient is placed in a position of comfort, but remains drowsy. EMT monitors vitals.]

- **BP:** 105/60 mmHg
- **Pulse:** 78 bpm
- **Respirations:** 14/min
- **SpO₂:** 96% on oxygen

[Patient is reassessed frequently for re-sedation, as opioids can outlast Narcan.]

Step 6: Notify Hospital & Transport

EMT (Radioing hospital):

"Medic 425 en route with a 35-year-old male found unresponsive with shallow breathing. Pinpoint pupils noted. Suspected opioid overdose. Administered 4 mg Narcan IN with respiratory improvement. BP 105/60, SpO₂ 96%. ETA 10 minutes."

[Patient is transported in a monitored position for continuous assessment.]

Scenario 8 - ODEMSA guidelines - Opioid Overdose

En Route

Dispatch: Medic 425 is responding to a **28-year-old female found unconscious in a motel room**. Caller reports **slow breathing and possible drug use**.

On Scene

Hotel Staff: *She checked in last night, and we found her unresponsive this morning. We saw some pills on the nightstand.*

[EMT and Paramedic approach the patient, who is lying on the bed, unresponsive, with shallow breathing.]

Step 1: Perform General Patient Assessment

EMT: *"Ma'am, can you hear me?"* **[Shakes shoulders, no response.]**

[EMT assesses for a pulse and breathing—pulse present, but respirations are 8 breaths per minute and shallow.]

EMT: *"We have an unconscious patient with inadequate respirations. Possible opioid overdose."*

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT opens the airway using the head-tilt/chin-lift maneuver and suctions the oropharynx as needed.]

EMT: *"Her airway is clear, but she's not breathing adequately. Let's assist with a BVM."*

[Bag-Valve Mask (BVM) is applied at 10-12 breaths per minute with supplemental oxygen.]

EMT: *"SpO₂ is 85%. Let's increase oxygen flow."*

Step 3: Obtain Blood Glucose Sample

[EMT checks blood glucose: 72 mg/dL (normal). Hypoglycemia is ruled out.]

EMT: *"Glucose is normal. We're dealing with an overdose."*

Step 4: Administer Naloxone (Narcan)

[EMT prepares Narcan for administration.]

EMT: *"Respirations remain inadequate. Administering 2 mg Narcan intranasally."*

[Narcan is administered via nasal spray. EMT continues ventilating with BVM.]

[After 2 minutes, the patient is still unresponsive. A second 2 mg dose is given.]

[Patient gasps and starts breathing more regularly.]

Step 5: Monitor and Prepare for Transport

EMT: *"Ma'am, can you hear me? You overdosed, but we gave you Narcan to help you breathe."*

[Patient is drowsy but responding. EMT monitors vitals.]

- **BP:** 100/58 mmHg
- **Pulse:** 70 bpm
- **Respirations:** 14/min
- **SpO₂:** 97%

[Patient is placed in a position of comfort and reassessed frequently.]

Step 6: Notify Hospital & Transport

EMT (Radioing hospital):

"Medic 425 en route with a 28-year-old female found unconscious with shallow breathing. Suspected opioid overdose. Received 4 mg Narcan IN with improved respirations. BP 100/58, SpO₂ 97%. ETA 12 minutes."

[Patient is transported under continuous monitoring for re-sedation.]

Scenario 9 - ODEMSA guidelines - Respiratory Distress

En Route

Dispatch: Medic 425 is responding to a **67-year-old male experiencing severe shortness of breath** at a community center. Caller reports **wheezing, labored breathing, and difficulty speaking**. The patient has a **history of COPD** and **uses home oxygen**.

On Scene

Bystander: *He was sitting down and suddenly started gasping for air. He's clutching his chest and can barely talk!*

[EMT and Paramedic approach the patient, who is in a tripod position, struggling to breathe.]

Step 1: Perform General Patient Assessment

EMT: *"Sir, my name is [Name]. We're here to help. Can you tell me what's going on?"*

Patient: *[Breathing heavily] Ca...can't...breathe...*

[EMT observes labored breathing, use of accessory muscles, and pursed-lip breathing. The patient is able to speak only in short phrases.]

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT checks for any airway obstruction—airway is clear, but patient is in significant distress.]

EMT: *"His airway is patent, but he has severe respiratory distress. We need oxygen immediately."*

[Pulse oximeter shows SpO₂ = 88%]

EMT: *"Administering oxygen via non-rebreather mask at 15 LPM to maintain SpO₂ above 94%."*

Step 3: Place Patient in a Position of Comfort

[Patient is placed in an upright seated position.]

EMT: *"Sir, stay seated upright. This will help your breathing."*

[Patient's breathing effort remains high. EMT continues assessment.]

Step 4: Assess Lung Sounds & Assist with Bronchodilator

[EMT listens to lung sounds—wheezing and diminished air movement heard bilaterally.]

EMT: *"Sir, do you have a prescribed inhaler?"*

Patient: *"Yes...in...my...pocket..."*

[EMT assists the patient with their metered-dose inhaler (MDI). If the patient does not have their inhaler, EMT prepares a nebulized treatment.]

EMT: *"Administering Albuterol 2.5 mg and Ipratropium 0.5 mg via nebulizer. Breathe in deeply through the mask."*

[Nebulizer is started, and patient is coached to take slow, deep breaths.]

Step 5: Monitor & Consider Additional Interventions

[SpO₂ improves to 91% after treatment, but respiratory distress persists.]

EMT: *"Still struggling. Let's prepare a second dose of Albuterol in 10 minutes if needed."*

[Vitals obtained:]

- **BP:** 155/88 mmHg
- **Pulse:** 110 bpm
- **Respirations:** 24/min (labored)
- **SpO₂:** 91% (after nebulizer)

[If symptoms worsen, EMT prepares for CPAP per protocol.]

Step 6: Notify Hospital & Transport

EMT (Radioing hospital):

"Medic 425 en route with a 67-year-old male in respiratory distress, history of COPD. Severe dyspnea, wheezing, accessory muscle use. Initial SpO₂ 88%, improved to 91% after nebulizer treatment. BP 155/88, HR 110, RR 24. Continuing Albuterol as needed. ETA 12 minutes."

[Patient is continuously monitored en route. If distress worsens, CPAP or further intervention is considered.]

Scenario 10 - ODEMSA guidelines - Respiratory Distress

En Route

Dispatch: Medic 425 is responding to a **52-year-old female experiencing severe shortness of breath** at a shopping mall. Caller reports **rapid breathing, wheezing, and difficulty speaking**. Patient has a **history of asthma and previous hospitalizations for respiratory issues**.

On Scene

Bystander: *She was walking through the store when she suddenly grabbed her chest and started gasping for air. She tried using her inhaler, but it didn't seem to help.*

[EMT and Paramedic approach the patient, who is sitting on a bench, visibly distressed and unable to complete full sentences.]

Step 1: Perform General Patient Assessment

EMT: *"Ma'am, my name is [Name]. We're here to help. Can you tell me what's going on?"*

Patient: *[Breathing heavily] I... can't... breathe...*

[EMT observes audible wheezing, accessory muscle use, and nasal flaring.]

Step 2: Assess and Support Airway, Breathing, and Circulation (ABC)

[EMT checks airway—airway is clear but breathing is labored and inefficient.]

EMT: *"Her airway is open, but she's struggling to breathe. We need oxygen immediately."*

[Pulse oximeter shows SpO₂ = 89%]

EMT: *"Administering oxygen via non-rebreather mask at 15 LPM to maintain SpO₂ above 94%."*

Step 3: Place Patient in a Position of Comfort

[Patient is assisted into a seated upright position to reduce work of breathing.]

EMT: *"Stay seated upright and try to take slow breaths."*

Step 4: Assist with Bronchodilator Treatment

[EMT checks for patient's prescribed inhaler.]

EMT: *"Ma'am, do you have an inhaler with you?"*

Patient: *"Yes...in my purse... but... it's not working!"*

[If inhaler is ineffective, EMT prepares nebulized treatment.]

EMT: *"Administering Albuterol 2.5 mg and Ipratropium 0.5 mg via nebulizer. Try to breathe it in slowly."*

[Nebulizer is started, and patient is encouraged to inhale deeply.]

Step 5: Monitor & Consider Additional Interventions

[SpO₂ improves to 92% after treatment, but respiratory effort remains high.]

EMT: *"She's still struggling. If needed, we'll administer a second dose of Albuterol in 10 minutes."*

[Vitals obtained:]

- **BP:** 145/85 mmHg
- **Pulse:** 122 bpm
- **Respirations:** 28/min (labored)
- **SpO₂:** 92% (after nebulizer)

[If distress persists, CPAP is considered.]

EMT: *"Ma'am, you're still having trouble breathing. We may use CPAP to help open your airways."*

[If patient deteriorates or becomes unresponsive, EMT prepares for assisted ventilation with BVM.]

Step 6: Notify Hospital & Transport

EMT (Radioing hospital):

"Medic 425 en route with a 52-year-old female in severe respiratory distress, history of asthma. Audible wheezing, increased work of breathing. SpO₂ 89% on arrival, improved to 92% after nebulized Albuterol/Ipratropium treatment. BP 145/85, HR 122, RR 28. Continuing Albuterol as needed. ETA 10 minutes."

[Patient is transported under continuous monitoring.]